SYNTHETIC TEST DOCUMENT - FICTIONAL PATIENT - NOT REAL MEDICAL DATA

CITY KIDNEY CARE - NEPHROLOGY CONSULTATION
Patient: Alicia Moreno DOB: 05/21/1995 Date: 08/14/2026
Consultant: Omar Haddad, MD (Nephrology)
Referred by primary care for protein and blood on a urine dipstick.

HISTORY: Annual physical in July showed protein and blood in the urine. No visible blood in urine, no urinary symptoms. Mild ankle puffiness by evening. Takes over-the-counter naproxen for joint aches a few times a week. Usual blood pressure in the 110s systolic.

VITALS: BP 136/86 (repeat 134/84) HR 84 Weight 61 kg

URINALYSIS (08/14/2026)	RESULT	REFERENCE
Protein	2+	Negative
Blood	2+	Negative
Red blood cells	15–20 /hpf	0–2 /hpf
Dysmorphic red cells	Present	Absent
Red cell casts	Few	None
White blood cells	0–2 /hpf	0–5 /hpf
Urine protein/creatinine ratio	0.9 g/g	<0.2 g/g

BLOOD (08/14/2026)
Creatinine 0.9 mg/dL (0.6–1.1) eGFR >90 Potassium 4.2 Albumin 3.6

ASSESSMENT: Glomerular-pattern hematuria with proteinuria below the nephrotic range; kidney filtration currently preserved.

PLAN: Start lisinopril 5 mg by mouth once daily for protein in the urine and blood pressure. Renal ultrasound. Repeat urinalysis, protein/creatinine ratio and potassium in 6–8 weeks. Serologic workup to be discussed at the follow-up visit (scheduled 10/30/2026). If findings persist, consider kidney biopsy.

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